

effective medicinal plants Native Americans had discovered—including well-known herbs such as echinacea (*Echinacea* spp., p. 92), goldenseal (*Hydrastis canadensis*, p. 105), and lobelia (*Lobelia inflata*, p. 110).

European settlers learned much from observing native practices. Over the course of the 19th and early 20th centuries, as pioneers moved west across the frontier territory, new plants were constantly being added to the official record of healing herbs. In addition to the three species mentioned above, about 170 native plants were listed in *The Pharmacopeia of the United States*.

Samuel Thomson and his Followers

Lobelia was one of the key herbs, along with cayenne (*Capsicum frutescens*, p. 74), advocated by Samuel Thomson (1769–1843), an unorthodox herbal practitioner who believed that all illness resulted from cold. His simple approach was entirely at odds with the conventional practices of his time (see North America, p. 50). Thomson's methods were often very effective and were well suited to the needs of people living in frontier territory. His system of medicine—in many ways an early form of naturopathy, in which ill health is treated with naturally grown food, sunlight, fresh air, and natural medicines—became extraordinarily popular, with millions of people across North America following his

methods. Thomson's success waned as other more sophisticated herbal approaches were developed—those of the Eclectics and Physiomedicalists, for example—in the fertile medical world of 19th-century America, which also saw the birth of osteopathy (a system of healing based upon the manipulation of bones) and chiropractic (a similar system primarily involving manipulation of the spine).

Western Influences on Asian Medicine

Across the world in China, Thomson's practices might have been looked on with a measure of surprise, but they would have been familiar. In Chinese medicine, there has always been a debate as to what degree illness arises from cold, and to what degree it arises from heat.



Ginseng has been used as a tonic remedy in Chinese medicine for at least 5,000 years.

The Shanghanlun (On Cold-Induced Maladies), written in the 2nd century CE, and revised and reinterpreted in commentaries over the last 1,800 years, recommends the herb cinnamon (*Cinnamomum* spp., p. 81) as a principal remedy when the patient “shivers with fever, breathes heavily, and feels nauseous.” In the 14th century, Wang Lu distinguished between cold-induced illness and febrile illness, and treated them in different ways, and this distinction was elaborated in greater and greater detail by different Chinese herbalists right up to the 19th century.

During the early 19th century, the influence of Western biomedicine was beginning to affect traditional practices in both China and India. This was certainly beneficial in many respects. The judicious incorporation of scientific principles and methods into traditional herbal healing offers the possibility of greatly refining the effectiveness of treatment.

But in India under British rule, Western medicine eventually became the only alternative. Ayurveda was seen as inferior to biomedicine (see India & the Middle East, p. 39). Western practice was introduced not as a complement to traditional medicine, but rather as a means to supplant it. According to one authority, “before 1835 Western physicians and their Indian counterparts exchanged knowledge; thereafter only Western medicine was recognised as legitimate and the Eastern systems were actively discouraged” (Robert Svoboda, *Ayurveda, Life, Health and Longevity*, 1992).

In China, the influx of Western ideas was less traumatic. Increasing numbers of Chinese medical students studied Western medicine, but this did not stop the continuing development of traditional herbal practice. By and large, each tradition was recognized as having both advantages and disadvantages.

Herbalism Outlawed 1850–1900

In Europe, conventional medicine was seeking to establish a monopoly for its own type of practice. In 1858, the British Parliament was asked to impose legislation banning the practice of medicine by anyone who had not been trained in a conventional medical school. Fortunately, this proposal was rejected, but in countries such as France, Spain, and Italy, and in some states of the U.S., it became illegal to practice herbal medicine without an orthodox qualification. Herbalists were forced to risk fines or imprisonment simply for providing herbal medicine to patients who had sought their help.

In Britain, concerns such as these, combined with a desire to establish Western herbal medicine as an alternative to conventional practices, particularly in the industrial cities of the North of England, led to the formation in 1864 of the National Institute of Medical Herbalists, the first professional body of herbal practitioners in the world. Its history is an example of how tenacious herbal practitioners have had to be simply to retain their right to give safe, gentle, and effective herbal medicines to their patients.